

2022 PM • 52:19

Interviewer 00:00

I will not be keeping the video I will just be using it for transcription.

Therapist 00:05

All right.

Interviewer 00:06

All right. And just before anything, I just want to get a better sense of your clinical practice as well, you primarily provide emotional focused therapy?

Therapist 00:14

No, actually, I primarily provide Integrated Behavioral Couples Therapy IBCT. I've been trained in emotional focus therapy in [Year], I believe in [Month Year]. So that's how I got my start into working with couples. Then I learned a bit more about CBCT cognitive behavioral couples therapy, and in the end made a switch to have a main focus on IBCT.

Interviewer 00:43

Interesting, why did you make that switch? Having been trained in that approach?

Therapist 00:48

So what I what I what I, I think it also I think, partially has to do with just a person that I am and the way I like to do therapy with clients. And I've also been trained like as a as a cognitive behavioral therapist in the past, and like, one of the things I think that most couple behavioral therapists do is give people a lot of homework. And in EFT the way I was trained initially, there was actually not that much homework you'd give to clients like the the idea was that through experiential experiences, within therapy, they would sort of generalize outside therapy. And it's not that black and white, there is some homework and you definitely try and have people work on, on the patterns that they're running into. But for example, I suppose it was also just wanted to give people some basic communication training, like in the sense of feedback skills, like how do you give feedback in a way that makes it more likely that the audience can hear it? And so some of those practical applications I found in CBCT. And like something else, like a thing called like an emotion thermometer, like, oh, it would be easier for people or partners to recognize when, like, for example, they were becoming more and more agitated, but also recognizing it in the other person. And what can you do when somebody's having certain at a certain level. So I was looking for that sort of practical addition on top of EFT, so I'm still used EFT a lot, but an added CBCT in it. And then at a certain point of time, I was teaching a course, like an introduction on couples therapy, and it wants to do sort of a broad perspective, like, Okay, what is actually being used in the field. And I ran across IBCT because it's like one of the three most evidence based forms of couples therapy, etc. And IBCT being- well, TBCT as well, traditional behavioral couples therapy, also known as behavioral marital therapy. And I really liked the the perspective on how things work and I what I liked is that it added like some of the elements that are being used in Act acceptance commitment therapy, into into the hall, if it feels like sort of all the elements of the different forms of therapy come together because they're still the practical part. Like they're still some problem solving skills communication training in there. But there's also looking at like those interaction patterns which are like very integral to EFT. And

then I like the the focus away from immediately focus on change, which is a little bit in BMT, but more for first, the focus back again on connection and acceptance. With the idea being that if you're able to get there, then the motivation for change becomes a lot larger, because if people are still entrenched in their own little bunkers, and just the other person, it's a lot harder to motivate them, motivate them for change. And also the nice thing is that if you have had them soften it a little bit, sometimes that change becomes less necessary.

Interviewer 04:39

Interesting, interesting journey. All right, so you, you are based in the [Country 1] have you always been based in the [Country 1]?

Therapist 04:46

Yeah, I mean, I've lived and worked in [Country 2] and in the [Country 3] as well, but I haven't been working as a clinician there.

Interviewer 04:58

And you primarily provide therapy to [Nationality] couples.

Therapist 05:03

yeah, primarily [Nationality] couples, I mean, sometimes International, but mostly [Nationality].

Interviewer 05:10

Okay, and so when clients come to you what usually is the presenting issue, and I know there's gonna be a broad range, but are there difficult? Reasons why they come to you?

Therapist 05:23

Hey, there's that I mean, there's many reasons like communication is one that pops up a lot. And it can be we don't communicate at all, or it can be that we always get into fights. Affairs are a pretty recurring theme. And sometimes it's sort of like, the emotional connection is gone. Like, I don't know whether I still feel anything for my partner, like, should we stay? Or should we just stop? There's a lot of other issues. Like, for example, if we're talking about, like, if I use communication as sort of a broad grouping, like, for example, differences in viewpoints around for example, child or bringing, or the household, I still use that umbrella thing, because the Yes, it is around how you raise the child. But it's very much also how you actually connect and communicate around that different that viewpoint.

Interviewer 06:30

Interesting. All right. And do you do a formal assessment when the clients come in?

Therapist 06:37

Yes.

Interviewer 06:38

And how did that happen?

Therapist 06:38

So right now, like for the for the past formal assessment consists out of one session with both partners, then an individual session with each partner, and then another conjoint session with both both partners, I oftentimes actually send a number of questionnaires to the partners before the first session. Yeah, so that's it from abroad.

Interviewer 07:13

Can you let me know what these questions are there?

Therapist 07:15

Sure. I use a questionnaire for attachment. And that's, I've been experiment a little bit whether I like the RQ more or the ECR like I use a simplified version of the NRV. It's a [Nationality] relationship questionnaire. One of the questionnaires which has actually pretty good psychometric qualities as opposed to a lot of questionnaires in the field I use let me see what are noted and I actually don't know the name by heart it's the one about? conflict scales. Let me just check because I thought I can find that pretty easily. And is, I am not one I'm sure what the actually, I'm not when I'm sure what questionnaire it is. Because I just have to live her literature references when it's on the questionnaires. So it's originally based on 19 A three questionnaire called a measure of styles of handling interpersonal conflict, but it has been adapted for couples in 2017, In a study by Dijkstra, Barelds, Ronner and and Nauta. And the study is called intimate relationships of the intellectually gifted: attachment style, conflicts style and relationship satisfaction among members of the Mensa society.

Interviewer 09:26

Perfect, I'd love to take a look at that.

Therapist 09:27

Yeah, let me I'll just dump the those links in the chat that way you have it.

Interviewer 09:37

Perfect. I wanted to ask, like, how informative do you find these scales attachment scale, the NRV the conflict style.

Therapist 09:46

Um, I find it somewhat informative. Like so I use it so. Um, underneath IBCT that are sort of a perspective on how we think that relationship issues arise. And it's called, like, it's in the analysis do what's called a deep analysis. It starts with starts with differences in personality, emotional sensitivities, external stressors, and patterns in communication or patterns in the interaction, and differences in personality. That's something I also assess during that, during the clinical interview, I would I have been considering using like something like a shortened version of the iPad and iPad Neo or something like that. It doesn't seem necessary, but it might be might be a nice addition. Emotional sensitivities, I feel like attachment style very much a connects to that. external stressors that's not in the questionnaires. That's something I use in the clinical interview, and patterns of communication there the conflict style, one is very useful. There's one other question or I use... it's... let me it's the c? I think primaria questionnaire, let me just check it real quickly, rather than. There's actually more that I used and just

said, I have problem areas questionnaire in. And then there's a questionnaire I use every session, and that's actually an adaptation. So it's a non validated questionnaire. It's an adaptation of the weekly questionnaire that is, in this website, let me also link that in the chat as well. I mean, just check that I'm saying the right thing. Alright, so what I use is a question five through 10. Where question five is a thing that I find very important because it actually is helpful in detecting intimate partner violence, which I otherwise don't always hear about from the partners themselves. And just because it's just, it's just one of the questions in a very short questionnaire. And then I adapted the first couple questions to the goals that I work on with IBCT. Let me just grab that as well. So question one is relationship satisfaction. It's all visual analogue scales. Sort of like how they use it in th?. How satisfied are you with your relationship? The second one is acceptance tolerance. Those are two goals that you're trying to two things that you're trying to improve in the IBCT. And the question there is, does your partner accept or tolerate you as a person? There's positivity, how much positivity is there in your relationship? Then there's communication how well is your communication your relationship, then there's problem solving? how able are you to solve everyday problems? And the last one is individual like how are you doing? Personally?

Interviewer 14:32

Through which questionnaire, this is the one length on the website.

Therapist 14:36

that's on the website, and it's an adaptation of the weekly questionnaire that's on that website. I see. All right, where I question five through 10 are the questions that I do use and the questions one through four are replaced by what I just said..

Interviewer 14:53

Interesting. All right. All right. And when you when you give these questionnaires, or do you have a any challenges when you are using these scales?

Therapist 15:03

None whatsoever.

Interviewer 15:06

And is it alright with you, we can ask how much these skills cost you?

Therapist 15:08

\$0.

Interviewer 15:12

Okay.

Therapist 15:15

These are all licensed free questionnaires. If I were to use actually the original version of the NRV, of last week last last dose relationship questionnaire that is copyrighted, but the Adapt adaptation of it, you're allowed to use for free.

Interviewer 15:38

Okay. And do you know how long like this whole battery of tests, usually the couples, how long the couples answer?

Therapist 15:48

I haven't asked them. But since they're all reasonably short questionnaires, not not too long.

Interviewer 15:56

All right. And so you asked this, when looking at your assessment procedure, the individual sessions that have conjoint sessions, where do you place these? Where do you make when you make them answer the questionnaires?

Therapist 16:09

I'd say 90% of cases, I would send questionnaires out before the initial interview before the end,

Interviewer 16:17

before the end for you go into—

Therapist 16:21

sometimes I do it after the initial one, if just has to do with the question that people are coming to see me for. But to use this one often, there's a specific form of therapy I provide called, like a relationship Boost, which is actually a single session therapy. It's a two hour long session. And then there's a six week follow up, where we actually have a conversation of 15 minutes or video call. And that is specifically for people who want to, who not don't necessarily have a lot of relationship issues, but they want to look at it very thoroughly see whether something they can improve is actually the goal is actually to make this a very positive and empowering experience. And I use a lot of the same questionnaires. One of the questionnaires that I haven't mentioned is called the I think the triangle or love scale, let me check. Yeah, it's called the triangle love scale by Sternberg, but also, again, a shortened version based on Feb. 2007, study by... I'll just link some stuff here. There are some text in there . And the idea of those sessions is not like, Alright, we're gonna look at everything that's wrong, but more like, we're going to look at your relationship from a lot of different perspectives. And it's up to you, what are you happy with? What you'd like to change. So for example, the triangle lovescale assesses like three aspects of relationship, commitment, passion and intimacy. And like, for example, there's quite a number of couples, I see where passion is reasonably low. And some of them do want to change it. And some of them are like, yeah, it's, it's alright, it's this phase where I am with kids, and it's fine. We don't want to work on that. But there's this other thing that we do want to improve. So and this seems like this is just like a fun addition.

Interviewer 18:54

Interesting.

Therapist 18:59

show so these are the most used questionnaires during the initial assessment. During couples therapy, I sometimes add other questionnaires or during an initial individual interview might, for example, use questionnaires which are screeners for individual psychotherapy or screeners for things like schema is like you using schema therapy, things like that.

Interviewer 19:31

Well, would you know, which exactly of the schema questionnaires you're using?

Therapist 19:36

Yeah, the schema questionnaire I use most often is the most recent version of the young schema questionnaire, the YSQ. I'm very happy with that because they reduce the number of questions from close to 200 to about 90. Were quite nice.

Interviewer 19:51

All right. And they wanted to ask you as well across the years and I believe this was in the email, as well that across the years, different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. But I wanted to get your view about it for you. What is relationship quality?

Therapist 20:13

That's a great question. So if I stay away from what researchers or other therapists say, and just define it as how I view relationship quality, I think relationship quality is the ability to actually keep reconnecting with your partner. It's, so it's about actually a certain kind of flexibility or adaptability. So like, they're sort of this, we feel like we're connected where, like, we're partners, we've got each other's backs, like, that's sort of a base for it for me. But it's alright, if something sometimes happens, that throws you off, and you're disconnect, and it can be a smaller or larger thing than being able to sort of reconnect again, that's, for me, what's part of good relationship quality?

Interviewer 21:10

Interesting. So for you, what differentiates couples with high relationship quality and versus low relationship quality is a certain sense of being able to reconnect with each other.

Therapist 21:24

Yes.

Interviewer 21:25

And would you say that, also that that is the most important thing in our relationship, the ability to keep reconnecting with your partner?

Therapist 21:35

I think so. Yes. I mean, there's so much that goes with that, like, for example, I know that in EFT, there's a lot of focus on that secure attachment and having that secure attachment, but I feel that even with insecure attachment, you can have a higher relationship quality. But it does require to you too, it helps if you understand, for example, the attachment style of yourself, the attachment style your

partner, what pitfalls can come with that. And if you're able to sort of just accept that, tolerate that cope with that, and then just keep making that connection, that's fine. You don't need to have that secure attachment, even though with the secure attachment, it's easier. Just like with that with like, if you have a personality disorder, like, well, it doesn't even matter which one, it brings its own challenges in the relationship. And the way that you both are able to connect, regardless of that, or keeping that into account is what for me to find relationship quality. So it's it relationship quality is for me about being able to deal with the challenges of being individuals, different individuals with different background and with different emotional sensitivities.

Interviewer 22:59

What, Why do you think some couples are, you know, why do you think some couples are able to reconnect with their partners more easily than others?

Therapist 23:11

I think there's several different factors? I mean, we can go back to the whole deep thing. So like, like, the closer personalities match, the often there's less friction or the more different you are as a person. I mean, you know, the saying like opposites attract, that's great and all but we also there's a lot of research that opposites struggled or in terms of relationship quality, relationship satisfaction. And it's easy to imagine like if I'm a very much an introvert, which I am and my wife is very much more an extrovert. You can get friction like around how often do we go to parties, just an example. Then there's the whole emotional sensitivities part like if you have a more emotional baggage and you're more sensitive to certain things, it's it's natural that it's a little harder. Like for example, if you have like any emotion sensors, not just like ski mask, personality disorders, and the attachment. It's also just like, if my previous partner has cheated on me had an affair is actual that I've become a little bit more sensitive to the idea that my partner is looking at different men. And then external circumstances, like if there's lots of stuff going on in my life or our life, it's easy that my window of tolerance is just a little smaller, like there's so much stress. I don't know about you, but I get a little bit more irritable. Like also if I have a lot lack of sleep. They've never don't sleep very well, I guess. I tend to be a little less forgiving and less flexible. And then the last like part like of the deep part like Pentacles communication, like, if I've never learned how to communicate in an effective manner, for example, around my emotions or my personal needs, then it's also normal that I get stuck there. So why do some couples have an easier time reconnecting than others? I think also partially just luck. Those things that feed into the challenges that you can have in a relationship, like I think, like, every single of those four themes that I just talked about, doesn't necessarily mean you'll have issues. But it's when everything sort of starts stacking on top of one another, it's more likely that you have those issues.

Interviewer 25:52

super interesting. And so you define relationship quality this way? How does this how you conceptualize relationships often differ from how clients can supply the relationship? And does this matter for therapy?

Therapist 26:07

No, that should just live. I'm kidding. I mean, it's, it's pretty, it's natural, bad. The way that actually clients define relationship, relationship quality is actually what's most important. Because I feel it is not

up to me to tell them this is how you should have a great relationship, the only thing I can do is say, these are the consequences of the things that you do and the way you interact. And then it's up to them what they want to do with it. Like, for example whether somebody wants to have a consensual non monogamous relationship or not, or wants to be in a monogamous relationship, I don't care. It's it's but what I do care about, for example, if it's consensual non monogamy, then I know that there are certain factors that make it more likely that they will succeed. And those are like communication, coercion and consensus, consensus reality. Like, it's very important to actually have communication about what do we feel is okay, and what do we feel is not okay, like, for example, you have a constitutional non monogamous relationship. But are Is it okay if I start dating friends? Or is it only people outside our circle or friends? Are all forms of sex sexual intercourse? Okay, or some forms are some forms aren't. And I think that's actually also important in regular relationships are monogamous relationships, I mean. But it's actually oftentimes more explicit in non consensual non monogamy, and the lack of coercion, like do you do what your partner was? Because you're afraid you will lose them otherwise?

Interviewer 27:59

How often do you get to handle non monogamous clients?

Therapist 28:03

Not as often as I'd like?

Interviewer 28:07

Ingesting and I'd like to ask you more about that later on. But I wanted to ask first, the relationship as you were talking about relationship as a complex system, it involves love, sex, identity, relationship, maintenance, conflicts, satisfaction and all that, given all of the scales that you were using before? How do you decide which one to focus on which one of this complex system to target?

Therapist 28:33

So there's a couple of things like, for me, there are sort of a couple of questions that sort of inform the way I try and support couples. The first one is actually around commitment. Are both partners actually still committed to a relationship and committed to trying to make it work? Or is one or of them already moving out of the relationship, or even both of them are moving out of the relationship? That I think that's the first important factor. And if both are committed, then I'm okay then then then it's easier to move forward. I'd say like regular couples therapy. And like in terms of commandment, it's okay, if you have no idea whether it can still work. But what I wanted to know is whether you're interested in discovering whether it can because let me try and help you figure out how as little as you want to try. Because if you don't want to try, like what users homework gonna be, for example. And then the second the second question, a cipher commitment. And actually, that's maybe the first question so commitment. Second question. First question, is there actually an actual crisis going on? And crisis in terms of an affair has just been discovered. My whole life is just like a roller coaster. How do I deal with this bar when one or both one somebody is suicidal? Because then the crisis needs to be addressed first. And then the third one is. So the commitment is high, there is not a crisis, then the third one is Is there actually other psychopathology going on. And where the suffering is so great that there's actually not enough space for working on the relationship? That's not often the case, I think I find very often you can do things parallel to one another. If all those three requirements are done, then I can actually

answer your question about how do my questionnaires help me inform what to focus on first. And to be honest, they don't really matter. And that has to do with the form of relationship counseling that I do, mainly IBCT, where if you virtually always first focus on acceptance, then tolerance, then positivity, then communication training, then problem solving. And there's a reason for that. Like, unless acceptance is already very high, then I can already then then I can move on to the next stage. But it's almost never that the acceptance is very high, if people see me for a couple of therapy, because most people don't come very quickly, they come after struggling for a couple of years. And but what the questioner is helping me with is presenting very understandable view of why they are struggling the way they do, and also connecting to you, okay, if this is what you're struggling struggling with. And this is the most problematic pattern, like for example, the struggle for autonomy versus connection, that's one that happens very often. And this is a problem area where it manifests a lot, for example, around raising their kids. Then, that will always be included in every exercise that we do within the couples therapy, like one of the first exercises you do around acceptance is called empathic joining, where you help people actually move from being able to talk about like, like, well, I call the hard or harder, harsher emotions like anger, frustration, irritation, those are all like very much direct towards the other person, the other person is wrong. And you can understand that if I tell you what you should do differently, you're less likely to accept my feedback in a very wholesome way, it's more likely that you're going to tell me what I am doing wrong if I am telling you what you are doing. But if I'm able to change into actually talking about the softer emotions, how the fact that you bought a car without even discussing it with me, actually makes me feel very alone in the relationship and makes the which makes me actually a little sad, because I want to feel that we're a team together. It's such a different conversation. And it's still painful, because sadness and fear are also painful emotions, but people are able to connect so much more around it. So if there was that struggle versus like autonomy, versus connection around around how we raise our kids, it's example around empathic joining would be concerning one of the incidents they've had around raising their kids.

Interviewer 34:26

So am I reading this write that the questionnaire is serve more really of a sort of like a rhetorical support event during therapy more than helping you in in in redirecting your intervention or indicating where the intervention should be? It's more for like to help the clients understand their issues, rather than in treatment planning with that.

Therapist 34:58

Yeah, I think you're so next end, like, again, like in the questionnaires, I see that there is a lot of acceptance and tolerance. That means that we do skip that initial phase, I just maybe check whether that's that's the thing. And in in the questionnaires, I find, for example, indicates for intimate partner violence, yeah, that's going to be the main focus of the therapy first. Other than that, it's really more, it's still a map for therapy. Because if I don't have that, I am not focusing on the most problematic type of interaction that they're having. And actually being able to break through that is actually very important for the therapy itself. And I'm maybe not using examples from the most problematic areas in their life can be child rearing. And it can be a religion, it can be a physical health, it can be the household, it can be anything.

Interviewer 36:05

Interesting. All right. And how can you determine when to end a couple of therapy?

Therapist 36:13

I mean, in there's a couple ways. One is the way I use IBCT, there, it's like a Dutch adaptation of the American original. It's actually just a 10 sessions protocol. So you do the initial assessment that's 4 sessions like two conjoined and two individual. And then there's 10 sessions. And basically, I said, like, there's five themes, acceptance, tolerance, positivity, communication, problem solving, every theme has two sessions, like, so that's safe. Like, if there's nothing really weird or big going on, you can actually just sail through like that. And then you have sort of a check, like, do people feel that they've learned enough and they can just like, try and keep improving on their own, then there's also that actually, sometimes people are done sooner. So if people feel that they have enough to go on, that's also a reason to end therapy. And a reason to not in therapy to actually work longer is like, for example, if somebody struggles around a certain theme a lot, some of them use a couple more sessions. Sometimes, it's very hard for people to actually work on therapy in their own time, then we do have a discussion whether it makes sense to have couples therapy, because I don't believe in in real lasting change if you don't have enough time to focus on relationships outside of therapy, because you see me once every couple of weeks. But there are exceptions where it's like the the life is the way it is, and me being there is at least better than nothing.

Interviewer 38:06

So usually are many of your cases, take 10 sessions. And how often are the sessions once every two weeks?

Therapist 38:18

yeah, in general, sometimes into between, like sometimes there are a little less frequent, definitely for moving towards the end of therapy, I try and space them out a bit more. So people have more time to work without me because I want them to need me less than less. And I sometimes have couples who actually do so much on their own. And I spaced their sessions out a bit more because I, I just had a lot of faith in their own way of actually working on the relationship. And that's actually very fun as well.

Interviewer 38:55

And how long are the sessions? One hour, one hour and a half?

Therapist 38:58

75 minutes.

Interviewer 39:00

Seventy five minutes? All right. So as you know, we currently are developing a relationship quality scale that better represents relationship as a complex system that would hopefully help therapists and treatment planning and evaluation. How interested would you be in this scale?

Therapist 39:19

I would be very interested.

Interviewer 39:22

Alright. And principally, what would you be looking for in a newly developed scale like this? Like, how can we make sure that this scale is going to be useful for your practice?

Therapist 39:32

So there's two actually, if I make simplify things, again, asked me about relationship quality. I actually am interested in two things relationship quality and relationship stability. And so if I use the questionnaire, as I like, to measure effect, those two things are interesting to me. Like, what do I see at the beginning of therapy? What do I see what I think is helpful? To find what do I see at the end? A would be other things that are important to me, there's actually that it's not connected to a certain therapy modality. So for example, if it were very attachment focused, even though I think attachment is important, I would be a little less interested in using a scale like that. If I were to use the I mean, if I want to see us in a more ? granular on a more granular level, there are aspects I'm interested in as well, like I'm interested in commitment. I mean, British, sure that's highly correlated to relationships, stability, but I mean, its commitment is important to me. Intimate partner violence is important to me. And that's, I actually say that for myself, but also for what I wish for the world. turns off that I noticed that there's actually a lot a couple of therapists who don't always assess for intimate partner violence. I actually hear colleagues say things like, yeah, no, that's something that doesn't happen in my in my practice, like, well, I know the numbers in the Netherlands are unfortunately pretty high. So if you never run across this practice, you're not screening correctly, or not screening at all.

Interviewer 41:34

All right, commitment. IPV. Anything else that you do on a wish list for you in a scale like this?

Therapist 41:46

I mean, it's really a, again, in terms of, is it for effect measurement, where, like, just the quality and stability or is enough for me? Or is it is am I going to be using it for other things, and then, like, for example, from an IBCT perspective, but it just said, I don't think the skills can be from a certain perspective, personality would actually be interesting. And emotional sensitivities would be interesting. external stressors would be interesting. communication patterns would be interesting.

Interviewer 42:22

All right. All right. All right. Sorry, I just want to be mindful of our time as well, because we booked for 45 minutes. Are you okay with going a little bit more? Or do you have...?

Therapist 42:31

I think so let me just make sure that is indeed the case. Yeah. All right. I can like use, definitely use 10 minutes.

Interviewer 42:42

Perfect. All right, me too. Okay, so we're planning to carefully develop and validate this scale over the course of about two more years after this. This is going to be a different price from what you're usually have in your room

Therapist 42:57

Like \$0

[Portion Redacted]

Interviewer 44:27

All right, and of course, you've already given your support, but I need to ask anyway, would you be interested to collaborate and continue your support in the development of?

Therapist 44:37

Absolutely, absolutely. I think I mean, even even if I feel that I'm happy with some skills I'm using, I feel that having a very well researched, scale, very carefully developed validated, checks for reliability. And I know that you're doing this also like across different cultures, which I think is very important is very interesting to me. Oh, actually something we did not talk about, like I do some research myself as well. And like one of the things that I am also trying to look into whether we we seem to presuppose that therapy works for all kinds of couples, with all kinds of, for example, sexual orientations. And I am kind of curious whether that is actually the case and so, a questionnaire that would also be for example, LGBT, LGBTQ plus. It would be important to me and would also allow for consensual non monogamous relationships.

Interviewer 45:46

Interesting. Yes. Okay, so just before we end I just like to have a quick review of what we've talked about we talked about getting a better sense of your clinical practice you primarily provide IBCT but you've been trained in EFT you've been trained in CBCT part of the switch really is you

Therapist 46:05

have not been trained in CB CT that's actually something I've took on myself.

Interviewer 46:09

Alright, so you like CB CT more than EFT before because of the well the assignments that you know, like it should generalize to outside of the lab sessions. But IBCT for you really is more of a who you are as a person because it feels like all the different elements and other modalities are there practical skills are focused on interaction pattern has focused and change, acceptance etc. So it has like ACT it has CBCT, DAS, EFT almost like that. Usually clients come to you communication problems affairs and the emotional connection is gone. And you do a formal assessment, you could do give out a roster of questionnaires before the intake attachment question as RQ UCAR. The NRV conflict styles are problem areas questionnaire and adaptation of the weekly questionnaire or triangular love scale. And during couple therapy screeners for individual psychotherapy and schema questionnaire, Did we miss anything from what you were using?

Therapist 47:11

No, I think in broadline sense. That's, that's pretty much right.

Interviewer 47:15

All right, and you do find these question is informative, you understand that it allows you to map out basically, both in treatment planning, because for example, if you would spot an IPV, then of course, that's what you're going to focus on, and also as to help clients understand the issue that they're having.

Therapist 47:38

And I do want to add to that, something I don't think I'm stressing is that it very much, it helps actually a lot in motivation for therapy, and it helps a lot in actually making the relationship issues feel less like they're failing as a person, we're failing as a couple more like, it is actually pretty natural that this happens. We can explain I mean, it still sucks. But it's it's very understandable and like that really helps both in motivation and also in actually getting people to feel some more hope and more perspective.

Interviewer 48:22

Perfect, alright. And then when asked about relationship quality for you, it really is about the ability to keep reconnecting with your partner to have that certain kind of flexibility. We talked about why you you think other couples have it easier in a way to to reconnect with each other. And and then we talked about whether the your clients conceptualize relationships differently for you, for you doesn't matter what matters really is their goals and how you can help them go towards their goals. And then we talked about how you actually target problem areas in any relationship for you before anything, there should be commitment. There should be no crisis there should be no other psychopathology. But for you, it's really the ABCT process acceptance for installer as positivity communication, then problem solving. And this usually

Therapist 49:21

just make sure I clarify one thing it's not that I don't do a cold start but if there's a crisis or there's a lack of commitment and all that, but it's more that I just addressed that issue first. It's not like go somewhere else is so it's like no, we're gonna address this and then we can do a couple survey.

Interviewer 49:36

Perfect so you Usually this takes 10 sessions once every two weeks, or yes, 75 minutes. And then you would be interested in for example, in effect measurement, relationship quality, relationship stability. And if it were to help in treatment planning, it would be about commitment ITV, possibly personality, emotional sensitivity, et cetera, et cetera 500 euros for you? Yes and no about about feasibility. But since you are part of the company, I would assume personally that yes, it will be might be for you. And as well as the new develop scale with

Therapist 50:24

the presuming it's also translated to Dutch, by the way, Oh.

Interviewer 50:27

definitely, I will planning to translate it at least 10 languages, hopefully, hopefully. And then, hopefully, the scale will be LGBTQ friendly as well, as well as with non monogamous couples. Now, before we get out,

Therapist 50:41

did you want to ask for different cultures?

Interviewer 50:45

Did you want to add anything to that?

Therapist 50:47

No, I'm gonna just a different culture thing.

Interviewer 50:49

All right, and different cultures. All right. Now before we end, I wanted to talk to you about how we will handle the data. Of course, we want our research to be as transparent and as verifiable by others as possible. So I wanted to know which of the data sharing options you're most comfortable with. One is that transcript questions can be shared, none of which can be identified to you. If you were bolder, another option is to share the whole transcript removed of overly sensitive and overly identifying details. So you will be anonymized. Although in this option, there is still a chance that a colleague who knows you professionally can piece together the details, like oh [Therapy] for [Nationality] couples might be. So there is still they might be able to narrow down who you are, although, again, no sensitive information will be shared. So among these two options, which one are you more comfortable with?

Therapist 51:43

I'm presuming that option two would be fine. I mean, I would like to see it before it goes out, but I'm, I'm I like research and data being shared. So I would like to support that.

Interviewer 51:59

Alright, so I will send you the transcript on these months. And then I'll get back to anything you want me to write. And thank you so much. I and this has been a great. I love this session. Thank you so much.

Therapist 52:13

Great. Yeah. I was curious. How have you had many interviews yet? Or

Interviewer 52:18

oh, I still have five. We sent a bunch of invitations. We have a pipeline scheduled Sorry, I'll just stop